

CITATION: Lakdawala v. Lipton, 2022 ONSC 7381

Court File No. CV-16-554230

DATE: 2022 05 20

**ONTARIO
SUPERIOR COURT OF JUSTICE**

B E T W E E N:

MOHAMED LAKDAWALA

Plaintiff

and

**AARON LIPTON, ERIN JOHNSTON, AVIVA INSURANCE COMPANY OF CANADA,
~~TAVERN COMPANY~~ 2439004 ONTARIO INC. operating as CAVA, ELKHOUND
INVESTMENTS INC. operating as SCALLYWAGS BAR AND RESTAURANT, JOHN
DOE 1, JOHN DOE 2, and JOHN DOE 3, JOHN DOE 4. JOHN DOE 5, and JOHN DOE 6**

Defendants

BEFORE: Associate Justice Ilchenko

COUNSEL:

Sirus Biniiaz, for the Defendants Aaron Lipton (“**Lipton**”) & Erin Johnston (“**Johnston**”)
(collectively, the “**Moving Defendants**”)

Sarah Naiman for Responding Party, Plaintiff Mohamed Lakdawala (the “**Plaintiff**”)

Lawrence Reimer for Defendant Elkhound Investments Inc. operating as Scallywags Bar
And Restaurant (“**Scallywags**”)

HEARD: May 9, 2022

RELEASED: May 20, 2022

ENDORSEMENT

I) Nature of Relief Sought by the Moving Defendants on Motion

[1] The Moving Defendants have brought an urgent motion to compel the Plaintiff to provide (and to continue to provide to trial) to the Moving Defendants answers to all allegedly outstanding undertakings and to compel the Plaintiff to attend additional physiatry and neurosurgical Defence Medical Examinations (a “**DME**” and collectively the “**DME’s**”) in this Action (the “**Action**”).

The Materials.

[2] In support of this Motion by the Moving Defendants, on April 27, 2022 Karen Mallia (“**Mallia**”), a law clerk in the offices of counsel for the Moving Defendants swore an Affidavit (the “**Mallia Affidavit**”).

[3] In response on May 3, 2022 Deanna Gilbert, counsel for the Plaintiff (“**Gilbert**”) swore a responding Affidavit (the “**Gilbert Affidavit**”).

[4] No cross-examinations took place with respect to the evidence in either the Mallia Affidavit or the Gilbert Affidavit.

[5] With respect to evidence of medical experts, no medical experts have sworn affidavits specifically for this motion, either in support of the Moving Defendants, or the Plaintiff.

[6] The Orthopaedic Report of Dr. Ford (the “**Ford Orthopaedic Report**”) obtained and served by the Moving Defendants, dated February 3, 2022 from an examination of the Plaintiff conducted on December 1, 2021 is attached as Exhibit EE to the Gilbert Affidavit, sworn in support of the Plaintiff. This report was not included in the Mallia Affidavit.

[7] The Physiatry Report of Dr. Sangha (the “**Sangha Physiatry Report**”) obtained and served by the Plaintiff, dated February 3, 2022 from an examination of the Plaintiff conducted on December 1, 2021 is attached as Exhibit E to the Mallia Affidavit, sworn in support of the Moving Defendants. This report was not included in the Gilbert Affidavit.

[8] To summarize, the Plaintiff’s Sangha Physiatry Report is an exhibit to the Moving Defendants’ Mallia Affidavit, and the Moving Defendants’ Ford Orthopaedic Report is an exhibit to the Plaintiff’s Gilbert Affidavit. In no case is either expert report made an exhibit to the affidavit of the respective expert that provided the report.

[9] I will deal with the evidentiary implications of these choices in greater detail below.

[10] The Orthopaedic report of Dr. McKee, obtained by the Plaintiff (the “**McKee Orthopaedic Report**”) is not in evidence, nor any other report. The CV’s of Dr. Fazl and Dr. Clark, the proposed Neurosurgical and Physiatry experts, respectively, for the Moving Defendants are exhibits to the Mallia Affidavit.

Glustein, J. Endorsement- April 8, 2022 (the “Glustein, J. Endorsement”)

[11] At a case conference held April 8, 2022 in this Action, and the related Action CV-16-566706, Glustein, J. set out the procedural context for this Urgent Motion:

“On October 28, 2021 I scheduled a 40 day trial for these two actions, which has been set for May 8, 2023 with a pre-trial on February 23, 2023. At that time, the defendants Lipton and Johnston (the “Lipton Defendants”) indicated that a Motion would be required to compel attendance at defence medical examinations and for answers to undertakings.

The Lipton Defendants did not schedule these motions promptly due to errors in dealing with the Associate Judges’ Motion Office. They are now advised that the Motion, even though less than two hours, cannot be scheduled until January 2023, only a few months before the scheduled May 2023 date.

The long scheduled 40 day trial date cannot be lost due to a lengthy scheduling delay for a brief and straightforward motion. Consequently, I order that the motion be scheduled by the Motions Office for the Associate Judges by not later than June 10, 2022 and counsel will be required to make themselves available on any date offered by the Motions Office during that period. If the Lipton Defendants do not immediately seek the motion date, with a request no later than April 12, 2022, they will be precluded from bringing the motion.

If there are any scheduling issues which are not resolved by my order, any party can immediately request a case conference with an Associate Judge or Administrative Associate Judge Brott, to be scheduled forthwith upon request. The 40 day trial cannot be lost.

Finally, I will remain seized of scheduling this matter if any further issues arise.”

[12] The “two actions” referred to in the Glustein, J. Endorsement are this Action and the companion action CV-16-566706 (the “**Tamanaha v. Lipton Action**”), which is scheduled to be heard together with this Action for 40 days in May of 2023, some detail of which I will provide below. I also heard an undertakings motion in the Tamanaha v. Lipton Action on consent at the same time as this Motion in this Action. These reasons do not determine issues in the Tamanaha v. Lipton Action.

[13] In accordance with the Glustein, J. Endorsement, this Motion, and the undertakings motion in the Tamanaha v. Lipton Action, were scheduled by the Motion’s Court Office to be heard before me on an urgent basis on May 9, 2022.

[14] I have proceeded to rearrange my schedule to determine this Motion as expeditiously as possible, and to provide this endorsement prior to the dates currently scheduled for the proposed DME's.

Specific Relief Sought

[15] The relief specifically sought on this Motion by the Moving Defendants is summarized in their Notice of Motion (the “**Notice of Motion**”):

“(a) An Order compelling the Plaintiff to provide to the Moving Defendants answers to all outstanding undertakings listed in Schedule "A", attached to this endorsement (the “**Undertakings**”);

(b) An Order that the Plaintiff attend and complete a neurosurgical medical examination with neurosurgeon Dr. Mahmood Fazl on June 13, 2022, from 9:00 a.m. to 11:00 a.m., at 2075 Bayview Avenue, Suite 138, in the City of Toronto, in the Province of Ontario, failing which the Moving Defendants may move, without notice to dismiss or discontinue the within proceeding as against them;

(c) An Order that the Plaintiff attend and complete a physiatry medical examination with physiatrist Dr. Benjamin Clark on June 14, 2022, from 9:30 a.m. to 11:30 a.m., at 5945 Airport Road, Suite 335, in the City of Mississauga, in the Province of Ontario, failing which the Moving Defendants may move, without notice, to dismiss or discontinue the within proceeding as against them;”

Preliminary Issue- request to bar Moving Defendants from bringing this Motion:

[16] As a preliminary issue, the Plaintiff objected to the Moving Defendants bringing the within motion at all alleging that the timing of the motion is in breach of the Court Ordered Timetable of Justice Glustein.

[17] On October 28, 2021, the parties attended a chambers appointment before Justice Glustein, at which he scheduled the trial date for May of 2023 and imposed a timetable which provided that the Defendants must bring any motions to compel the Plaintiff to attend defence medical examinations, or to answer undertakings, by February 28, 2022.

[18] On February 8, 2022, in paragraphs 52-59 of the Gilbert Affidavit counsel for the Plaintiff alleges that they became aware that the Moving Defendants were requesting an urgent Chambers Appointment due to difficulty scheduling these motions with the Court, but they allege that the real difficulty was that counsel for the Moving Defendants had allegedly waited until Christmas night more than 2 months after the Order was issued, to request a motion date, and, that they kept e-mailing the wrong email in the Motions Court office. They allege that their availability for motion dates had not been discussed prior to become aware of this request by the Plaintiff for a chambers appointment, and they had not been copied with communications with the Court regarding the scheduling attempts.

[19] The Plaintiff alleges that as a result this Motion was initially scheduled for January 17, 2023 (four months prior to the start of trial), but that Glustein, J. intervened and Ordered this motion to be scheduled and heard by June 10, 2022, but allegedly left this preliminary issue open.

[20] The Mallia Affidavit has no evidence on this issue and it is not dealt with in the Factum of the Moving Defendants. In argument counsel for the Moving Defendants advised that it was implicit in the Glustein, J. Endorsement that the time period for bringing these motions had been extended by the requirement that they be scheduled and heard by June 10, 2022.

[21] As I advised counsel, I was not at the case conference that gave rise to the Glustein, J. Endorsement, but on the reading of that Endorsement, set out verbatim above, I can find nothing that supports the Plaintiff's contention that Glustein, J. left it open to me to determine whether I should hear this Motion as a result of the alleged breach of the Case Timetable established by Glustein, J.

[22] The best person to determine whether the Moving Defendants were in breach of the timetable ordered by Justice Glustein was Justice Glustein.

[23] Given that the time limits placed by Justice Glustein in the Glustein, J. Endorsement on the Moving Defendants to:

- 1) obtain this hearing, and

- 2) the requirement ordered by Glustein, J. that this Motion be heard by an Associate Justice prior to June 10, 2022, rather than January of 2023 as was apparently originally scheduled,

I cannot see how the Plaintiff can argue that the time limit for bringing this motion was not extended, and the hearing of this motion today was not effectively Ordered, by Glustein, J., and I have no authority whatsoever to vary or set aside the Order of a Superior Court Justice.

[24] I therefore heard this Motion.

Disposition of Undertakings Motion:

[25] The nature of the relief sought by the Moving Defendants with respect to the Undertakings allegedly outstanding is not the usual relief sought on an undertakings motion. At the hearing I was asked to deal with 8 specific Undertakings, which are summarized in the chart produced by the Moving Defendants for this Motion, which I have attached as Appendix A to this Endorsement, with my disposition for each.

[26] With respect to Undertakings 4 and 7, the issue was not that no documents were sent by counsel for the Plaintiff in answer to the undertakings given. The Plaintiff's position was that they sent the documents they had undertaken to provide relating to the clinical Notes and Records of Dr. Khan and Dr. Adam. The Moving Defendants stated that they did not receive

them. Counsel for the Plaintiff agreed to resend the documents, forthwith. That was a sensible solution.

[27] The remainder of the alleged unfulfilled undertakings dealt with the issue of the ongoing nature of the undertakings to provide documents to the date of trial, currently scheduled for May of 2023. As noted in the chart, the Plaintiff has produced documents in response to each of these undertakings, and in some cases provided updated documents in March and April 2022 to documents previously provided in 2019, 2020 and 2021.

[28] However what the Moving Defendants are seeking is an order compelling the Plaintiff to continue updating this documentation to Trial. That was not an order that I was willing to grant, without evidence that the Plaintiff was in breach of its obligations for continued discovery, and R.31.07 and R.31.09 relating to Undertakings and correcting incomplete answers, of which there was none.

[29] With respect to Undertaking 1, counsel for the Plaintiff has agreed to provide a copy of the new passport of the Plaintiff, if a new passport has been obtained prior to trial, and has confirmed they will fulfill that undertaking. No order compelling is necessary.

[30] With respect to Undertaking 2, counsel for the Plaintiff has agreed to provide a copy of the 2021 tax return, when that copy is filed and assessed and that they will continue to fulfill the undertaking to provide tax returns, as they have already provided for the 2012-2020 tax years. No order compelling is necessary.

[31] With respect to Undertaking 3, counsel for the Plaintiff has provided correspondence on April 12, 2022 confirming that there are no further records from St. Michaels Hospital for the period September 23, 2020 to March 10, 2022, with records prior to that date having been produced by the Plaintiff in 2019 and 2021, but agreed to request further records if their client reattends. No order compelling is necessary.

[32] With respect to Undertaking 5, counsel for the Plaintiff has agreed to write again to Toronto Rehab forthwith to request any further records for the period after July 2, 2021. No order compelling is necessary.

[33] With respect to Undertaking 6, counsel for the Plaintiff has agreed to write again to Dr. Barbera forthwith to request any further records for the period after June 30, 2021. No order compelling is necessary.

[34] With respect to Undertaking 8, counsel for the Plaintiff has agreed to write again to OHIP forthwith to request an updated OHIP Summary for the period after October 26, 2021. No order compelling is necessary.

[35] I note that I have not compelled the Plaintiff to take any of these steps, because Judicial compulsion was not necessary, but the Plaintiff agreed to do so, confirming the Plaintiff's duties for continued discovery and the duty to continue to complete the Plaintiff's undertakings by updating information, but I have not found the Plaintiff to be in breach of those duties.

[36] That disposes of the Undertakings Motion.

II) Factual Context in which the Motion to compel further Defence Medical Examinations is Brought

The Collision

[37] The Plaintiff alleges that on June 9, 2015, he suffered severe injuries after being involved in a motor vehicle collision (the “**Collision**”). The short fact scenario is that the Plaintiff was the driver of a Beck cab that was allegedly struck by a vehicle driven by the Moving Defendant Lipton, and owned by the Moving Defendant Johnston. The Defendant Scallywags allegedly served alcohol to the Defendant Lipton, as allegedly did the other restaurant Defendant Cava, that did not appear on this Motion.

[38] The Plaintiffs in the companion Tamanaha v. Lipton Action were the passengers in this Plaintiff’s, Lakdawala’s, cab, and this Plaintiff is a Defendant in the Tamanaha v. Lipton Action, along with these Moving Defendants Lipton and Johnston. Because of these factual connections, this Action, and the Tamanaha v. Lipton Action, are being heard together for 40 days in May of 2023.

The Damages Claimed by the Plaintiff

[39] In the Amended Statement of Claim issued on June 7, 2016 the Plaintiff sought damages in the amount of \$3,000,000 and claims the following with respect to damages that resulted from the Collision:

“...

20. As a result of the crash, Mr. Lakdawala suffered permanent serious impairments of important physical, mental, and/or psychological functions and/or permanent serious disfigurements including, but not limited to: a reduced level of consciousness; cognitive impairment; fractures through his left C-6 vertebrae involving the left pedicle, facet, and lamina with displacement; a fracture through the left facet and transverse process of C7; a comminuted fracture of the right femur; a fracture of the left first rib; bilateral pulmonary contusions; a right ear laceration; soft tissue injuries to his neck and back; pain; restricted range of motion; reduced sitting, walking, and standing tolerances; weakness; muscular atrophy; disturbed sleep; fatigue; grotesque and obvious scarring; and psycho- emotions dysfunction. These injuries, impairments, and/or disfigurements have been accompanied by profound pain, suffering and a loss of enjoyment of life.

21. As a further result of the crash, Mr. Lakdawala has been and/or will be forced to participate in extensive rehabilitative therapy; to consume significant quantities of medication; to rely upon ambulatory and assistive devices; and/or to undergo invasive medical procedures including, but not limited to, open reduction and internal fixation of his right femoral fracture.

22. Mr. Lakdawala has been and will be put to hospital, medical, attendant care, rehabilitative, and/or other out-of-pocket expenses; the value and cost of which he claims. The particulars of Mr. Lakdawala's special damages claims will be provided prior to the trial of this action.

23. Mr. Lakdawala pleads that any payments he has received to the date of trial for medical, rehabilitative, attendant care, housekeeping, income replacement, visitor's expenses, and/or damage to clothing were for goods and/or services reasonably required and incurred as consequence of the crash, from the point of the crash to the date of trial. e to clothing were for goods and/or services reasonably required and incurred as consequence of the crash, from the point of the crash to the date of trial.

24. As a further result of the crash, Mr. Lakdawala has been unable to engage in his social, marital, athletic, caregiving, recreational, and other activities of daily living to the same extent that he was able to prior to the crash. These losses have dramatically reduced and will continue to reduce Mr. Lakdawala's enjoyment of life.

25. As a further result of the crash, Mr. Lakdawala has suffered a loss of income. Mr. Lakdawala will continue to suffer a loss of income, a loss of competitive advantage, and/or a loss of earning capacity in the future. The particulars of Mr. Lakdawala's past and future loss of income claims will be provided prior to the trial of this action.

26. As a further result of the crash, Mr. Lakdawala has been and will be unable to engage in his usual housekeeping and home maintenance chores to the same extent that he was able to prior the crash. Mr. Lakdawala has been and will be forced to rely upon the assistance of other, to incur expenses, to be indebted to others, to perform these tasks less efficiently, and/or to forego performing these tasks at all. The particulars of Mr. Lakdawala's past and/or future housekeeping and home maintenance claims will be provided prior the trial of this action.

...

Plaintiff's Evidence on requests for Further DME's

[40] The Plaintiff states in the Gilbert Affidavit that on January 18, 2017 and February 28, 2017 the Plaintiff served the medical-legal Sangha Physiatry Report, dated November 30, 2016 and McKee Orthopaedic Report, dated February 1, 2017.

[41] With respect to the request by the Moving Defendants for Physiatrist and Neurosurgical DME's the Plaintiff provides the following evidence in the Gilbert Affidavit relating to the request by the Moving Defendants to compel a Physiatrist DME:

"...6. By letter dated January 18, 2017, a true copy of which is attached hereto and marked as Exhibit "B", Mr. Lakdawala served the medical-legal report of physiatrist, Dr. H. Sangha, dated November 30, 2016.

7. By letter dated February 28, 2017, a true copy of which is attached hereto and marked as Exhibit "C", Mr. Lakdawala served the medical-legal report of orthopaedic surgeon, Dr. M. McKee, dated February 28, 2017

8. In June of 2018, the parties scheduled a mediation returnable for June of the following year 2018 Discovery of Mr. Lakdawala

9. On November 29, 2018, Mr. Lakdawala was examined for discovery; true excerpts of which transcript are attached hereto and marked as Exhibit "D" (Qs. 180, 204, 233-236, 459, 485-492). During his examination, defence counsel was alerted to the fact that Mr. Lakdawala spends approximately 4-6 months a year in India. Mr. Lakdawala testified:

(a) He spends his time between Canada and India on account of having family and a family business in India.

(b) He takes two trips to India per year, staying for 2-3 months at a time.

(c) In 2018, he was in India from January to March, then again from May to September.

2020 Action Set Down following Mediation

10. On June 11, 2019, the mediation of this action and that of the companion action failed.

11. On March 4, 2020, Mr. Lakdawala set this action down for trial.

2020 Request for Psychiatry & Orthopaedic DMEs

12. By letter dated October 29, 2020, attached to the Moving Defendants' Motion Record as Exhibit "G", the Moving Defendants initiated their first request for any DME. The letter requested that Mr. Lakdawala attend a DME with physiatrist, Dr. B. Clark, on December 8, 2020. The letter further advised that the Moving Defendants would also be scheduling other DMEs, but did not identify any specialties, names, or dates.

13. By letter dated November 6, 2020, attached to the Moving Defendants' Motion Record as Exhibit "H", the Moving Defendants requested that Mr. Lakdawala also attend a DME with orthopaedic surgeon, Dr. P. Marks, on January 14, 2021. By separate letter later that day, the Moving Defendants advised that the DME had been re-scheduled to January 12, 2021.

14. By letter dated November 6, 2020, attached to the Moving Defendants' Motion Record as Exhibit "I", Mr. Mandel advised that Mr. Lakdawala would be in India from November 25, 2020 to June 15, 2021. Mr. Mandel suggested that

one or both of the DMEs be re-scheduled to take place prior to November 25, 2020. I am advised and do verily believe that Mr. Mandel heard nothing further.

...

26. After Trial Scheduling Court, by email to me dated June 23, 2021, counsel for the Moving Defendants ("Mr. Biniaz") advised that the DMEs with Dr. Marks and Dr. Clark, respectively, had been re-scheduled for July 15 and September 14, 2021. Attached to the Moving Defendants' Motion Record as Exhibit "N" is a true copy of Mr. Biniaz's e-mail dated June 23, 2021.

27. By email dated June 23, 2021, I advised that Mr. Lakdawala remained in India due to the Government of Canada's (then) flight ban from India due to Covid-19, which ban was set to expire on July 21, 2021. Consequently, the July 15, 2021 DME with Dr. Marks was subsequently cancelled by the Moving Defendants. Attached to the Moving Defendants' Motion Record as Exhibit "O" is a true copy of my e-mail dated June 23, 2021.

28. Between June 24-29, 2021, the parties (primarily our office and that of counsel for the Moving Defendants) exchanged emails discussing a revised scheduled for the exchange of expert reports. Our office proposed a DME report deadline of January 31, 2022 so as to preserve the option of a Spring 2022 trial date, whereas the Moving Defendants maintained their June 30, 2022 position. Attached hereto and marked as Exhibit "L" is a true copy of this e-mail exchange."

Plaintiff's Evidence on request for Neurosurgical DME:

[42] The Plaintiff has not served a neurosurgical expert report and it is the evidence of the Plaintiff in the Gilbert Affidavit that the Plaintiff has not been treated by a neurosurgeon as a result of his collision related injuries since November of 2015.

[43] The Gilbert Affidavit states the following in relation to the Plaintiff's Neurosurgical treatment:

"29. During the course of the June 29, 2021 section of the above-referenced email chain, within hours of the scheduled case conference, Mr. Biniaz advised for the first time that the Moving Defendants would also be seeking to have Mr. Lakdawala undergo a "neurological assessment." After I pointed out that Mr. Lakdawala had neither been treated nor assessed for medical-legal purposes by a neurologist, Mr. Biniaz advised that it was, in fact, a neurosurgical DME that was being sought. Attached hereto and marked as Exhibit "M" is a true copy of this e-mail chain.

30. Mr. Lakdawala has not served a neurosurgical expert report.

31. Mr. Lakdawala has been seen by neurosurgery at St. Michael's

Hospital in a treating capacity; however, the last neurosurgical consultation note related to the subject collision dated November 19, 2015, a true copy of which is attached hereto and marked as Exhibit "N", stated that he does not require any follow up at the Neurosurgery Clinic.

32. While Mr. Lakdawala continued to be seen by the Head Injury Clinic at St. Michael's Hospital, this was in relation to a right frontal tumor that was found in the course of the investigation of his motor vehicle collision related injuries. Moreover, a progress summary note dated June 18, 2019, a true copy of which is attached hereto and marked as Exhibit "O", confirmed that from a clinical perspective, Mr. Lakdawala remains completely asymptomatic (in relation to the tumor).

33. In any event, the fact that Mr. Lakdawala had (albeit, rarely) been seen by a neurosurgeon in a treating capacity has been known to the Moving Defendants since January 23, 2019 when they were served with the updated clinical notes and records from St. Michael's Hospital. Attached hereto and marked as Exhibit "P" is a true copy of the serving letter dated January 23, 2019.

June 2021 Judicial Case Conference & Continued DME Issues

34. On June 30, 2021 (re-scheduled from June 29th at the Court's request), the parties appeared before the Honourable Mme. Justice D. Wilson at the case conference during which Her Honour:

- (a) Expressed discontent with the late-requested DMEs, notwithstanding the India complication;
- (b) Would not provide the parties with pre-trial or trial dates until they had agreed upon a Timetable for the Service of Expert Reports;
- (c) Instructed counsel for the Moving Defendants to advise the Plaintiffs (in both companion actions) of the proposed schedule for DMEs;
- (d) Instructed the parties to work out a Timetable for the Service of Expert Reports, and to advise Her Honour once that had been done.

35. By email to Mr. Biniaz dated June 30, 2021, following the case conference, I suggested that the Moving Defendants first advise of the schedule for their proposed DMEs so that the parties could then discuss the timing for the delivery of those and any reply reports. Attached hereto and marked as Exhibit "Q" is a true copy of my e-mail dated June 30, 2021.

36. By email to Mr. Biniaz dated July 6, 2021, I followed-up to my email of June 30, 2021 as I had yet to receive any further DME notices. Mr. Biniaz replied that a DME schedule would be provided "shortly". Attached hereto and marked as Exhibit "R" is a true copy of Mr. Biniaz's e-mail dated July 6, 2021.

37. By letter to Mr. Kazdan dated August 11, 2021, Mr. Mandel indicated that in view of the Government of Canada's extension of the Covid related flight ban from India to September 21, 2021, the physiatry DME with Dr. Clark returnable September 14, 2021 would have to be re-scheduled unless it could be done virtually. I note that to that point, in the six weeks since the case conference had occurred, the Moving Defendants still had not provided any schedule for the other DMEs. Attached to the Moving Defendants' Motion Record as Exhibit "R" is a true copy of Mr. Mandel's letter dated August 11, 2021.

38. Between August 13-14, 2021, Mr. Biniaz and Mr. Mandel exchanged emails regarding the scheduling of the DMEs given the flight ban. Mr. Mandel proposed that the DMEs take place virtually with an in-person update that could be considered, if necessary, upon Mr. Lakdawala's return. Attached to the Moving Defendants' Motion Record as Exhibits "S" and "r" is a true copy of this e-mail exchange.

The October 2021 Chambers Appointment & Orders

39. By email to Mr. Biniaz dated August 16, 2021, a true copy of which is attached hereto and marked as Exhibit "S", I expressed concern that it had been over six weeks since Her Honour had instructed the Moving Defendants to provide a schedule for its proposed DMEs and that there were still none scheduled.

40. By email to counsel dated September 1, 2021, a true copy of which is attached hereto and marked as Exhibit "T", my law clerk ("Ms. Running"), requested counsel's availability for a chambers appointment to address the outstanding scheduling issues since the deadline was approaching for our action to be dismissed from the trial list if no trial date was set.

41. By email to the Court dated September 9, 2021, a true copy of which is attached hereto and marked as Exhibit "U", Ms. Running submitted a chambers appointment request to the Court. The chambers appointment was scheduled for October 28, 2021.

42. By letter dated October 27, 2021 (incorrectly listed as 2020, though sent in 2021) a true copy of which is attached to the Moving Defendant's Motion Record as Exhibit "V", the Moving Defendants advised that:

- (a) The orthopaedic DME had been re-scheduled to December 1, 2021 with Dr. Ford instead of Dr. Marks;
- (b) The physiatry DME had been re-scheduled to April 5, 2022 with Dr. Clark;
- (c) The neurosurgical DME was not yet scheduled.

43. By reply email of the same date, attached to the Moving Defendants' Motion Record as Exhibit "W", I advised that Mr. Lakdawala:

- (a) Would attend the DME with Dr. Ford;
- (b) Would not attend the DME with Dr. Clark, returnable on a day five months out, particularly given judicial commentary in the case law regarding Dr. Ford's ability to assess chronic pain;
- (c) Would not attend the DME with this yet-to-be identified or scheduled neurosurgical expert, particularly given that Mr. Lakdawala had not retained any such expert.

44. Further, within my email of October 27, 2021, I stated (my emphasis added): "Should your client be serious about moving forward with this [neurosurgical] defence medical, then we would encourage your office to devote its time to promptly bringing a motion rather than to scheduling an assessment we indicated four months ago our client would not attend."

[44] The Consult Note from Dr. Vaidyanath at the Head Injury Clinic at St. Michael's Hospital at Tab "O" to the Gilbert Affidavit states:

"Mr. Mohamed Lakdawala was seen in followup [sic] in the head injury clinic at St. Michael's Hospital today. I have been following him to ensure stability of a right frontal tumor. This was found incidentally in the course of investigation of his traumatic brain injury symptoms from his MVA on June 9, 2015. He has now-had 3 MRIs of the brain. The latest one in September 2018 revealed stability of the right frontal tumor with a differential remaining DNET versus other Low- grade glial tumor, but there was an area of susceptibility in the right parietal lobe, which had not been commented on previously, corresponding to a small focal hemorrhage.

From a clinical perspective, Mr. Lakdawala remains completely asymptomatic.

At this stage, I would like to seek the opinion of one of our neurosurgeons, Dr. Sunit Das, to review the scans and let us know if there is any reason to be concerned and if any followup [sic] monitoring of the tumor is required."

Moving Defendants' Evidence on requests for Further DME's

[45] The Moving Defendants in the Mallia Affidavit paint a different picture of their requests for further DME's:

"5. On November 29, 2018, the plaintiff attended at an Examination for Discovery.
6. Since then, counsel for the plaintiff, Mr. Sloan Mandel of Thomson, Rogers Barristers and Solicitors ("Mr. Mandel"), has submitted to our office various medical records and reports on behalf of his client, including:

(a) The clinical notes and records of St. Michael's Hospital, which document numerous visits made by the plaintiff to its neurosurgery department, along with consultations with neurosurgeons Michael Cusimano and Sunit Das, and psychiatrist Chantal Vaidyanath, from 2015 to 2020 in connection with complaints related to a traumatic brain injury secondary to the subject Accident; and

(b) a Psychiatry Assessment Report dated November 30, 2016, wherein psychiatrist Harpreet Sangha discusses the plaintiff's various limitations that she attributes to the Accident, and also references a traumatic brain injury for which the plaintiff was being followed by Dr. Vaidyanath. Attached hereto and marked as Exhibit "D" are the true copies of the relevant portions of the clinical notes and records of St. Michael's Hospital.

Attached hereto and marked as Exhibit "E" is a true copy of Dr. Sangha's Psychiatry Assessment Report dated November 30, 2016.

...

REFUSAL TO ATTEND AT TWO MEDICAL EXAMINATIONS

12. On October 29, 2020, I wrote to Mr. Mandel and advised that our office had made arrangements for the plaintiff to attend a defence medical examination with psychiatrist Benjamin Clark ("Dr. Clark") on December 8, 2020. Attached hereto and marked as Exhibit "G" is a true copy of my correspondence dated October 29, 2020.

13. On November 6, 2020, I wrote to Mr. Mandel and advised that in addition to the medical examination with Dr. Clark, our office had made arrangements for the plaintiff to attend a medical examination with orthopaedic surgeon Paul Marks ("Dr. Marks") on January 12, 2021. Attached hereto and marked as Exhibit "H" is a true copy of my correspondence dated November 6, 2020.

14. On November 6, 2020, Mr. Mandel wrote to our office and advised that the plaintiff "will be out of the country for the period spanning November 25, 2020 through to June 15, 2021 to attend to family matters in India ... [and] will therefore not be attending upon the above appointments (unless either one or both could be bumped forward to take place prior to November 25, 2020)." Attached hereto and marked as Exhibit "I" is a true copy of Mr. Mandel's correspondence dated November 6, 2020.

15. I was unable to "bump forward" either the psychiatry or the orthopaedic medical examinations to take place sometime prior to the plaintiff's travel to India on November 25, 2020. Accordingly, I re-scheduled the psychiatry medical examination with Dr. Clark to take place on September 15, 2021, and the orthopaedic examination with Dr. Marks to take place on July 15, 2021. Attached hereto and marked as Exhibits "J" and "K" are true copies of the confirmations of examinations with Drs. Clark and Marks, respectively.

16. While the plaintiff was seemingly still in India, Mr. Mandel delivered on his behalf a Trial Record, and on June 4, 2021, wrote to us to advise that our attendance was required at Trial Scheduling Court on June 23, 2021 for the purposes of setting this matter down for Trial. Attached hereto and marked as Exhibit "L" is a true copy of Mr. Mandel's correspondence dated June 4, 2021.

17. On June 17, 2021, I emailed Mr. Mandel's office and inquired as to whether the plaintiff had returned to Canada on June 15, 2021 as scheduled so that we could proceed with the rescheduled defence medical examinations. Attached hereto and marked as Exhibit "M" is a true copy of my correspondence dated June 17, 2021.

18. On June 23, 2021, the parties - including Mr. Sirus Biniiaz of our office ("Mr. Biniiaz") and Ms. Deanna Gilbert of Thomson, Rogers Barristers and Solicitors ("Ms. Gilbert") - attended at Trial Scheduling Court before the Honourable Madam Justice Wilson. I am told by Mr. Biniiaz, and I verily believe it to be true, that at said appearance, Justice Wilson directed the parties to attend at a to-be-scheduled Case Conference before her, prior to which the parties were to confirm among themselves (i) the length of time required for the Trial of this action; and (ii) the deadlines for delivering any expert reports.

19. Following the attendance at Trial Scheduling Court, on June 23, 2021, Mr. Biniiaz wrote to Ms. Gilbert and inquires as to whether the plaintiff had returned to Canada and, if not, the date on which he was scheduled to do so. In his correspondence, Mr. Biniiaz also advised Ms. Gilbert that our office had scheduled orthopaedic and psychiatry medical examinations with Drs. Marks and Clark on July 15 and September 14, 2021, respectively. Attached hereto and marked as Exhibit "N" is a true copy of Mr. Biniiaz's correspondence dated June 23, 2021.

20. On June 23, 2021, Ms. Gilbert advised our office that the plaintiff was "indeed, presently in India." Ms. Gilbert noted that the Government of Canada had imposed a travel ban on flights from India until July 21, 2021, subject to further extensions, and, as such, "suggest[ed]" that we re-schedule the orthopaedic medical examination with Dr. Marks. Ms. Gilbert further informed us that Dr. Marks was her "cousin-in-law" and left it to our office to select another expert. Attached hereto and marked as Exhibit "O" is a true copy of Ms. Gilbert's correspondence dated June 23, 2021.

21. On June 24, 2021, Ms. Gilbert wrote to all parties and reiterated that pending the termination of the travel ban on July 21, 2021, she "anticipate[d] that [the plaintiff] should be able to attend [the psychiatry medical examination with Dr. Clark on September 14, 2021] and a re- scheduled orthopaedic defence med[ical] examination." "Assuming that [said medical examinations] go ahead in and around September of 2021," Ms. Gilbert insisted that the defendants deliver their expert reports by January 31, 2022. Attached hereto and marked as Exhibit "P" is a true copy of Ms. Gilbert's correspondence dated June 24, 2021.

22. On June 24, 2021, Mr. Biniiaz responded to Ms. Gilbert's correspondence and expressed that upon his eventual return to Canada, the plaintiff would be required to attend three defence medical examinations, the third being a neurosurgical examination. In light of the uncertainty regarding the plaintiff's eventual return date to Canada, Mr. Biniiaz proposed that the defendant's deadline for delivering expert reports be set at June 30, 2022. Attached hereto and marked as Exhibit "Q" is a true copy of Mr. Biniiaz's correspondence dated June 24, 2021.

23. On August 11, 2021, Mr. Mandel wrote to our office and advised that the Government of Canada's travel ban on passenger flights from India had been extended to September 21, 2021, such that the plaintiff could not attend an in-person psychiatry medical examinations with Dr. Clark on September 14, 2021, prompting our office to then cancel same. Attached hereto and marked as Exhibit "R" is a true copy of Mr. Mandel's correspondence dated August 11, 2021.

24. On August 13, 2021, Mr. Biniiaz wrote to all parties to advise that our office would cancel Dr. Clark's medical examination, and to request that the plaintiff reconsider its position to require delivery of expert reports by January 31, 2022. Attached hereto and marked as Exhibit "S" is a true copy of Mr. Biniiaz's correspondence dated August 13, 2021.

25. On August 14, 2021, Mr. Mandel responded to Mr. Biniiaz and expressed concern about proceeding with a neurosurgical medical examination "given that [the plaintiff] has not been treated by a neurosurgeon for years." Mr. Mandel further proposed that medical examinations be conducted virtually in an effort to avoid delay in scheduling a Trial. Attached hereto and marked as Exhibit "T" is a true copy of Mr. Mandel's correspondence dated August 14, 2021.

26. On October 18, 2021, Mr. Mandel's office wrote to all parties to advise that the plaintiff had returned to Canada. Attached hereto and marked as Exhibit "U" is a true copy of correspondence from Thomson, Rogers Barristers and Solicitors dated October 18, 2021.

27. On October 27, 2021, I wrote to Mr. Mandel and advised that our office had made arrangements for the plaintiff to attend the following defence medical examinations:

(a) an orthopaedic examination with orthopaedic surgeon Michael Ford ("Dr. Ford") on December 1, 2021; and

(b) a psychiatry examination with Dr. Clark on April 5, 2022.

Attached hereto and marked as Exhibit "V" is a true copy of my correspondence dated October 27, 2021.

28. On October 27, 2021, Ms. Gilbert responded to me and advised that while she was agreeable to have the plaintiff attend an orthopaedic examination with Dr. Ford on December 1, 2021, she was not agreeable to have her client submit to either a psychiatry or a neurosurgical examination. This constituted the first instance in which counsel for the plaintiff opposed a psychiatry medical examination. Attached hereto and marked as Exhibit "W" is a true copy of Ms. Gilbert's correspondence dated October 27, 2021.

29. On December 1, 2021, I wrote to Mr. Mandel and Ms. Gilbert and advised that our office had made arrangements for the plaintiff to attend the following defence medical examinations:

(a) a neurosurgical examination with neurosurgeon Mahmood Fazl on January 24, 2022; and

(b) a psychiatry examination with Dr. Clark on April 5, 2022.

Attached hereto and marked as Exhibit "X" is a true copy of my correspondence dated December 1, 2021.

30. That same day, Ms. Gilbert responded to me and reiterated her position that the plaintiff would not be attending either a psychiatry or a neurosurgical examination. Attached hereto and marked as Exhibit "Y" is a true copy of Ms. Gilbert's correspondence dated December 1, 2021."

[46] The parties have presented some diametrically opposed interpretations of the same interactions, and these facts must be applied to the legal principals under which additional DME's may be ordered.

III) Law and Analysis: Compelling Defence Medical Examination

[47] All underlined text in these reasons is emphasis added by me for these reasons.

[48] The Court has considered all materials and arguments raised by the parties on this Motion. Any failure by the Court to refer in these reasons to specific arguments and materials raised does not reflect that the Court has not considered those arguments.

Legislation and *Rules of Civil Procedure*

[49] [Section 105](#) of the [Courts of Justice Act \(Ontario\)](#), RSO 1990, c C.43, provides that the court may order a party to undergo an examination by one or more health practitioners where the physical or mental condition of a party to a proceeding is put in question. The section reads:

105 (1) In this section,

“health practitioner” means a person licensed to practise medicine or dentistry in Ontario or any other jurisdiction, a member of the College of Psychologists of Ontario or a person certified or registered as a psychologist by another jurisdiction. R.S.O. 1990, c. C.43, s. 105 (1); 1998, c. 18, Sched. G, [s. 48](#).

Order

(2) Where the physical or mental condition of a party to a proceeding is in question, the court, on motion, may order the party to undergo a physical or mental examination by one or more health practitioners.

Same

(3) Where the question of a party’s physical or mental condition is first raised by another party, an order under this section shall not be made unless the allegation is relevant to a material issue in the proceeding and there is good reason to believe that there is substance to the allegation.

Further examinations

(4) The court may, on motion, order further physical or mental examinations.

Examiner may ask questions

(5) Where an order is made under this section, the party examined shall answer the questions of the examining health practitioner relevant to the examination and the answers given are admissible in evidence. R.S.O. 1990, c. C.43, s. 105 (2-5).

[50] R.33.02 states:

33.02(1) An order under [section 105](#) of the [Courts of Justice Act](#) may specify the time, place and purpose of the examination and shall name the health practitioner or practitioners by whom it is to be conducted.

33.02(2) The court may order a second examination or further examination on such terms respecting costs and other matters as are just.

[51] The Moving Defendants cite McCartney, J's decision in *Vellunga v Nelmes* [2000] O.J. NO. 4579, 3 C.P.C. (5th) 360 stating that the purpose of s. 105 of the CJA as:

"4 The purpose of this section is to ensure that if a party raises an issue as to his or her medical condition, the allegation can be fairly contested by the other side."

[52] Both the Moving Defendants and the Plaintiff agreed with me that Master Pope's decision in *Godin v. Goncalves*, 2014 ONSC 7297 ("**Godin**") generally set out the tests for the ordering of a DME. Master Pope states:

"[24] The purpose of defence medical examinations is to put the parties on equal footing by allowing the defendant to meet the case advanced by the plaintiff and to respond to the allegations made by the plaintiff in the statement of claim: see *Girao v. Cunningham*, 2010 ONSC 4607, [2010] O.J. No. 3642, at para. 18, citing *Chapell v. Marchall Estate*, [2001] O.J. No. 3009.

[25] In *Tsegay v. McGuire*, 1 C.P.C. (5th) 311, 25 C.C.L.I. (3d) 317 (ONSC), at para. 5, Gillese J. stated that medical reports are critical to the resolution of personal injury disputes, the choice of physician to conduct the defence medical examination is a matter of importance to the defence and that medical reports contribute to settlements and constitute the crucial expert evidence on which a court relies to do justice between the parties if the matter proceeds to trial. Her Honour went on to state that the policy behind the defence medical exam is to uphold the right of the defendant to conduct his or her defence and to assist the court at trial by furnishing expert evidence that is subject to the adversarial process.

[26] The defendant relies also on *Girao* at para. 23 for the proposition that where both the physical and mental condition of the plaintiff are in question, the defendant should, subject always to reason and good sense, have the opportunity to have the plaintiff assessed in both aspects of the plaintiff's condition, and that s. 105(2) clearly contemplates examinations by more than one health practitioner when required.

[27] Justice Mitrow in *Ramrup v. Lazzara*, 2014 ONSC 130, 58 C.P.C. (7th) 417, at para. 46, considered a motion by the defendants to require the plaintiff to attend two additional defence medicals. He emphasized that the purpose of a second or subsequent defence examination is not to go "one for one" or "tit for tat" with the number of plaintiff expert reports. It is not simply a numbers game. The issue of trial fairness concerns a defendant having an adequate opportunity of meeting the plaintiff's case.

[28] The defendants submit that a defendant has the prima facie right to select the physician to perform the examination to which it is entitled: see *Smith v. Liberty Life Assurance Company of Boston* (2003), 37 C.P.C. (5th) 357, 2003 CanLII 11628 (ONSC), at para. 14.

[29] The theme running through the jurisprudence is whether a further defence medical is necessary as a matter of fairness in order to level the playing field. There is no dispute that the test to be applied in determining whether to order a further defence medical is necessity, fairness and prejudice: see *Ramrup*, at para. 47, citing *Jeffrey v. Baker*, [2010] O.J. No. 4415 (S.C.J.), at para. 12. 7297 (CanLII)

[30] The plaintiff relies on the general proposition held in *Fehr v. Prior* (2006), 40 CP.C. (6th) 381, [2006] O.J. No. 5244, at para. 5 (S.C.J.) that where the court held that as long as a person's physical or mental condition is relevant to a material issue in the proceeding, a party is entitled to compel that person to undertake one examination by a health care practitioner. Any further examinations must either be undertaken with the consent of the party to be examined or with leave of the court in its discretion.

[31] The plaintiff relies on the amendments to the Rules effective January 1, 2010 that emphasize proportionality in litigation. The general principle in interpreting the rules set out in subrule 1.04(1) requires that the rules be liberally construed to secure the just, most expeditious and least expensive determination of every civil proceeding on its merits. Subrule 1.04(1.1), subtitled "Proportionality", was added which states: In applying these rules, the court shall make orders and give directions that are proportionate to the importance and complexity of the issues, and to the amount involved, in the proceeding.

[32] The plaintiff also cites from Coulter A. Osborne, Q.C., *Civil Justice Reform Project: Summary of Findings and Recommendations*, November 2007, c. 9, at p. 68, upon which the Rule amendments were based, wherein Justice Osborne found that there was general agreement that the increased use of experts is a factor that increases the cost of litigation and causes delay through trial adjournments.

[33] The plaintiff also relies on the *Evidence Act*, R.S.O. 1990, c. E.23, s.12 and the policy underlying that provision which is to permit a party to call more than three experts only where the time and expense involved is justified: see *Jones v. Spencer* (2005), 16 C.P.C. (6th) 349, [2005] O.J. No. 1539, at para. 15 (S.C.J.).

[34] The court in *Jones*, took note that the defence had proposed five defence medicals at a time when no expert had examined the plaintiff and identified medical issue beyond that expert's expertise or expressed any disagreement with anything in the plaintiff's case. The court opined that in those circumstances, the defence considered it routine and appropriate to engage a host of experts in every allegedly "catastrophic" personal injury case. That approach was viewed by the court as inconsistent with the statutory scheme, wasteful of private and public resources, and should be discouraged."

[53] Associate Justice McGraw in *Chandrababu v. Tharmalingham*, 2021 ONSC 4885 also set out the general rules in determining whether to order a party to attend a defence medical examination after a matter has been set down for trial, post *Godin*:

"[20] A defendant's right to respond to a plaintiff's expert report is a substantive right involving principles of trial fairness (*Bonello v. Taylor*, 2010 ONSC 5723 at para. 15). The Court of Appeal has long recognized that defence

medical assessments are an integral part of the discovery process (*Bellamy v. Johnson*, (1992) 1992 CanLII 7491 (ON CA), 8 O.R. (3d) 591 (C.A.) at para. 16).

[21] The purpose of a defence medical examination is to achieve trial fairness and a level playing field by putting the parties on a basis of equality as near as it is possible (*Ortiz* at paras. 2-5; *Rohit* at paras. 24-29; *Vanderidder v. Aviva Canada Inc.*, 2011 CarswellOnt 8811 at para. 35). Allowing a plaintiff to adduce expert evidence of damages while denying the defendant the ability to test and challenge that evidence through its own expert is inconsistent with trial fairness, unduly restricts the defendant's pre-trial preparation and risks creating a perceived imbalance (*Moore v. Wakim*, 2010 ONSC 1991 at para. 4; *Vanderidder* at para. 35)."

Moving Defendants' additional jurisprudence on general test for ordering DME's

[54] The Moving Defendants in their factum cite *Godin*, as well as *LaForme v Paul Revere Life Insurance* 2006 CanLII 81803 (ON SCDC) at 14 and *Klinck v Dorsay* 2021 ONSC 6285 ("**Klinck**") as stating that DME's are necessary for the purposes of:

"(a) promoting fairness in the litigation process, or "level[ing] the playing field" by providing the defendant an opportunity to meet the case advanced by the plaintiff and respond to the allegations that have been raised as against him or her;

(b) uphold[ing] the defendant's right to conduct its defence as advised; and

(c) assist[ing] the Court at an eventual Trial by furnishing crucial expert evidence upon which the trier of fact may rely to ensure the proper administration of justice."

[55] The Moving Defendants in their Factum summarize the findings of Ryan-Bell, J. in *Klinck* in applying the "...test to be applied in determining whether to order a further defence medical is necessity, fairness and prejudice" from *Godin*:

"23. When considering the "necessity" arm of the *Godin* test, the Court, in *Klinck*, held that the moving party must establish that the requested examination is likely to produce information relevant to a medical issue in the action. With respect to the "fairness" arm, the Court considered the seriousness of the plaintiff's alleged injuries and how substantial his claim would be if proven. Finally, in considering the "prejudice" arm of the *Godin* test, the Court observed that there is an overarching societal interest in securing the just, most expeditious and least expensive determination of a civil claim. The Court then held that this societal interest outweighs the plaintiff's concerns regarding the onerousness of undergoing an examination."

[56] The Moving Defendants also cite Broad, J.'s decision in *Arsenault-Armstrong v Burke and Solcan* 2014 ONSC 2759 as an example of a situation where an DME was ordered after the Court found that the plaintiff had obtained a number of reports that commented upon his psychological injuries whereas the defendant was not in possession of

any responding reports. I note however in that case (which was decided several months before *Godin*) that Broad, J. also stated:

“[14] It was observed by Master Haberman in the case of *Nelson v. Thiruchelvan* [2005] O.J. No. 743 (Master), at para. 21, that multiple defence medical examinations are not granted as a matter of course and that the general principle that emerges from the cases is that the moving party must provide evidence that addresses why they seek particular examinations and the basis for seeking that relief must be clear and compelling.

[15] In the case of *Gravelle v. Pearson* [2001] O.J. No. 281 (S.C.J.) Valin, J. cited the decision of Master Sandler in *Trotter v. Cattin* (1977), 15 O.R. (2d) 800 (Master) for the proposition that a second or further medical examination is not a right but a discretionary matter and that the defendant must, on such an application, "present sufficient evidence to persuade a court as to the necessity for a second or further physical examination."

Plaintiff's additional jurisprudence on general test for ordering DME's

[57] The Plaintiff responds that with respect to the general principles for the granting of a further DME, that there is no unfettered right to a DME and that the onus on the moving party is a heavy one, citing Ray, J. in *Babcock v. Destefano*, 2016 ONSC 5352 (CanLII).

[58] The Plaintiff argues that given the intrusive nature of DME's, the onus rests with the Moving Defendants to establish that:

- (a) An opinion from a particular specialty is needed; and
- (b) An in-person assessment for that expert to render an opinion (i.e. as opposed to a paper review assessment).

citing *Wadie Narouz v. Desjardins Financial Security*, 2015 ONSC 540 (CanLII) and purportedly *Campbell v. Cotton*, 2002 CanLII 45423 (ONSC), citing the decision of Master Dash at paragraph 27 (however the Plaintiff links in this Factum to the CanLII decision for the Divisional Court decision of Dunnet, J in *Campbell v Cotton* approving the decision of Master Dash which has only three paragraphs, but not the actual Master Dash decision, that does not appear to be in CanLII– however paragraph 27 of Master Dash's decision is cited as follows later in the Plaintiff's Factum):

“... There is no evidence before me that an examination of the plaintiff is necessary in order for Ms. Vellone to prepare a vocational assessment or a future care cost analysis. There is nothing to prevent the defendant from commissioning and serving its own vocational assessment or future care cost analysis. What is prevented is compelling the plaintiff to undergo an examination for that purpose. Nothing would prevent Ms. Vellone from reviewing the extensive records and reports already generated, critiquing the reports of Mr. Katz and Ms. Straub, and preparing her own assessment. The onus is on the party requesting the examination to provide evidence that an examination is necessary for an

assessment and report. I have no evidence whatsoever that an examination of the plaintiff is necessary in order for Ms. Vellone to prepare her reports. In fact I have no evidence from Ms. Vellone whatsoever, other than her curriculum vitae.”

[59] The Plaintiff also cites case law regarding timing and prejudice of the DME’s but I will deal with those issues specifically in applying the general *Godin* test principles in determining whether to order a further DME of “...necessity, fairness and prejudice.”

Interpretation of Rules and Proportionality

[60] Rule 1.04 of the Rules of Civil Procedure reads:

Interpretation

General Principle

1.04 (1) These rules shall be liberally construed to secure the just, most expeditious and least expensive determination of every civil proceeding on its merits. R.R.O. 1990, Reg. 194, [r. 1.04 \(1\)](#).

Proportionality

(1.1) In applying these rules, the court shall make orders and give directions that are proportionate to the importance and complexity of the issues, and to the amount involved, in the proceeding. O. Reg. 438/08, s. 2.

Matters Not Provided For

(2) Where matters are not provided for in these rules, the practice shall be determined by analogy to them. R.R.O. 1990, Reg. 194, [r. 1.04 \(2\)](#).

[61] It is of assistance to reproduce the entirety of Perell, J.’s reasoning on the issue of Proportionality in paragraphs 154 to 164 of in *Ontario v. Rothmans Inc.*, 2011 ONSC 3685 (S.C.J.) (“*Rothmans*”), regarding proportionality in discovery:

G. The Role of Proportionality

“[154] The recent amendments to the *Rules* added new Rule 29.2 (Proportionality in Discovery).

[155] Rule 29.2.03 requires the court to consider a variety of factors associated with the idea of proportionality when making a determination of whether a party or other person must answer or produce a document in an affidavit of documents, for an examination for discovery, for an inspection of property, for a medical examination or for an examination for discovery by written questions. The rule states:

CONSIDERATIONS

General

29.2.03 (1) In making a determination as to whether a party or other person must answer a question or produce a document, the court shall consider whether,

- (a) the time required for the party or other person to answer the question or produce the document would be unreasonable;
- (b) the expense associated with answering the question or producing the document would be unjustified;
- (c) requiring the party or other person to answer the question or produce the document would cause him or her undue prejudice;
- (d) requiring the party or other person to answer the question or produce the document would unduly interfere with the orderly progress of the action; and
- (e) the information or the document is readily available to the party requesting it from another source.

Overall Volume of Documents

(2) In addition to the considerations listed in subrule (1), in determining whether to order a party or other person to produce one or more documents, the court shall consider whether such an order would result in an excessive volume of documents required to be produced by the party or other person.

[156] It is notable that the proportionality principle for discovery is not made to apply to cross-examinations, which suggests that the rule makers appreciated that cross-examinations were not meant to be a substitute for the discovery process, where the proportionality principle definitely has an important role to play.

[157] In the case at bar because the Master essentially treated the cross-examination as if it were an examination for discovery or because he regarded proportionality as relevant to his determination of the scope of proper questions and undertakings for a cross-examination on an affidavit filed for a very important motion, he considered the proportionality principle to be applicable. As noted above, he quoted rule 1.04 (1) and new subrule 1.04 (1.1.), which state:

General Principle

1.04 (1) These rules shall be liberally construed to secure the just, most expeditious and least expensive determination of every civil proceeding on its merits.

(1.1) In applying these rules, the court shall make orders and give directions that are proportionate to the importance and complexity of the issues, and to the amount involved, in the proceeding.

[158] In my opinion, the Master erred in principle in his approach to the proportionality principle.

[159] The proportionality principle is a manifestation of the policy of frugality that led to the introduction of the simplified procedure to the *Rules of Civil Procedure*. To use a metaphor, the normal *Rules of Civil Procedure* are the Cadillac of procedure, an expensive vehicle with all the accessories. However, not all actions or applications require such an expensive vehicle, and a Chevrolet, a serviceable, no frills vehicle, will do just fine for many cases, and it will provide access to justice and judicial economy.

[160] Proportionality is a parsimonious principle. In *Javitz v. BMO Nesbitt Burns Inc.*, 2011 ONSC 1322 at para. 28, Justice Pepall noted that the proportionality principle was introduced because the system of justice was under severe strain because cases were taking too long and costing too much for litigants. In the passage quoted by the Master from Chapter 5 of Lord Woolf's report, Lord Woolf said that his overall aim was to "improve access to justice by **reducing** the inequities, cost, delay, and complexity of civil litigation." In *Abrams v. Abrams*, 2010 ONSC 1928 at para. 70, Justice D.M. Brown, stated: "Proportionality signals that the old ways of litigating must give way to new ways which better achieve the general principle of securing the "just, most expeditious and least expensive determination of every proceeding on its merits."

[161] In the case at bar, however, because of his concern about Lord Woolf's ideal of an "equality of arms," and because of the strategic importance he gave to the jurisdictional motion, the Master concluded that proportionality could have an expansive influence and thus the jurisdictional motion in an action with an enormous claim called for a Rolls-Royce of procedure, where the court should have as much relevant information as possible, as complete a record as is available, and all reasonably available relevant evidence regardless of its age or the difficulties associated with finding it.

[162] In adopting this approach, the Master departed from his own views, with which I agree and would endorse, expressed in *Warman v. National Post Co.* [2010 ONSC 3670](#) (Master), where he stated at paras. 84-86:

[84] The time has come to recognize that the "broad and liberal" default rule of discovery has outlived its useful life. It has increasingly led to unacceptable delay and abuse. Proportionality by virtue of the recent revisions has become the governing rule. 84. To the extent that there remains any doubt of the intention of the present rules I see no alternative but to be explicit.

85. Proportionality must be seen to be the norm, not the exception - the starting point, rather than an afterthought. Proportionality guidelines are not simply "available". The "broad and liberal" standard should be abandoned in place of proportionality rules that make "relevancy" part of the test for permissible discovery, but not the starting point.

86. If embraced by the courts, parties and their counsel, such proportionality guidelines offer hope that the system can actually live up to the goal of securing for the average citizen "a just, speedy and inexpensive determination" of his or her case

[163] In my opinion, an expansionary approach to proportionality is wrong. A parsimonious proportionality principle provides a useful tool for cases large and small. The base line is that the *Rules of Civil Procedure* are designed for cases of all sizes, but the proportionality principle allows the court to downsize the procedure and still do justice for the parties. If downsizing is not procedurally fair then the normal rules should apply to the proceedings without augmentation.

[164] If adopted as a precedent, the Master's approach of treating proportionality as having an expansionary influence destroys the parsimony of the proportionality principle and allows the argument that because a case is important or the claim large, there should be more procedure not less procedure. The proportionality principle would lose its utility for large cases, such as class proceedings and other public interest litigation, where justice can be done by reducing not expanding the procedure. The proportionality principles yields an "equality of arms" by arms reduction and is not meant to prompt an arms race. In my opinion, in this particular case, the Master erred in principle in his treatment of the proportionality principle."

Evidentiary issue regarding attaching of Experts Reports to Affidavits on Motion

[62] As I alluded to in the introduction to the evidence on this Motion, no experts have sworn affidavits on this Motion. All of the evidence of the Moving Defendants has been introduced through Mallia, a law clerk, significant portions of which must be on information and belief, and without the ability to provide any expert medical evidence whatsoever. Similarly, the Plaintiff's evidence is of Gilbert, a lawyer.

[63] As noted in the reasons of Ryan Bell, J. in *Klinck*, cited by the Moving defendants on this Motion:

"[30] Second, the plaintiffs' motion materials include a number of physicians' reports attached as exhibits to a law clerk's affidavit. To the extent the plaintiffs purport to rely on these reports as expert reports on this motion, they are not properly before the court. Attaching these reports as exhibits to a law clerk's affidavit does not amount to proof of the contents of the reports: *Tomson v. Jackson*, 2021 ONSC 3422, at para. 55, citing *Peirson v. Bent* (1993), 1993 CanLII 8559 (ON SC), 13 O.R. (3d) 429 (S.C. Gen.

Div.). Plaintiffs' counsel submitted that had the moving parties advised him of their concerns, he would have made these physicians available for cross-examination. The moving parties were under no obligation in this regard. The articles from medical journals in the plaintiffs' materials are also not properly before the court on these motions."

[64] Paradoxically, in each case, the witnesses Mallia and Gilbert attach the experts reports obtained by the other party to their affidavits to support their own cases. Attaching these opposing party reports to the Gilbert Affidavit and the Mallia Affidavit "...does not amount to proof of the contents of the reports" as stated by Ryan Bell, J.

[65] No party objected however to their use in evidence on this Motion. I will deal with the specifics of this issue below.

Analysis: Request for Physiatry DME:

[66] The two requested DME's have different factual histories, and different legal considerations so I will apply the *Godin* factors of Necessity, Fairness and Prejudice, and the R.1.04 and *Rothmans* Proportionality tests separately to these requests.

Factual matrix for request for Physiatry DME:

[67] From the voluminous evidence of the chronology of the request for the Physiatry DME, and attached correspondence in the Gilbert Affidavit and the Mallia Affidavit I find the following facts as relevant to my determination:

- 1) The Sangha Physiatry Report was issued by Dr. Sangha on November 30, 2016, after an assessment conducted on November 23, 2016 and sent to counsel for the Responding Defendants on January 18, 2017, before mediation and discoveries took place;
- 2) Dr. Sangha in his CV attached to the Sangha Physiatry Report states that he is certified by the Royal College of Physicians and Surgeons of Canada in Physical Medicine and Rehabilitation, is a Diplomat of the Canadian Society of Clinical Neurophysiologists in EMG (Dip. CSCN - EMG), has additional fellowship training in Pain Medicine from the University of Toronto, Department of Anesthesia, and holds a full- time clinical appointment at Toronto Rehabilitation Institute, a University of Toronto Teaching and Research Hospital, where he has dual appointments on both the Musculoskeletal Rehabilitation Program and Neuro-Rehabilitation services;
- 3) An unsuccessful mediation was conducted on June 11, 2018;
- 4) The Examination for Discovery of the Plaintiff was conducted on November 29, 2018;
- 5) This Action was set down for Trial by the Plaintiff on March 4, 2020;

6) The first request by the Moving Defendants for a Physiatry DME was on October 29, 2020;

7) The Plaintiff was out of the country from November 25, 2020 until September, 2021, in part prevented from returning to Canada by Government of Canada COVID travel restrictions;

8) The Plaintiff, through counsel, first refused to attend Physiatry and Neurosurgical DME's on October 27, 2021 but agreed to attend the Orthopaedic DME. Several physiatry DME dates had been scheduled and cancelled prior to this date, seemingly without dispute by counsel for the Plaintiff, other than the fact that the Plaintiff was stuck outside of Canada;

9) The Orthopaedic DME agreed to by the Plaintiff occurred on December 1, 2021 and the Ford Orthopaedic Report was issued on February 3, 2022 and served on or about April 7, 2022;

10) Dr. Ford comments on the Sangha Physiatry Report in the Ford Orthopaedic Report;

11) Dr. Ford's CV from the Ford Orthopaedic his states that he is an orthopaedic spine and trauma surgeon on active staff at Sunnybrook Health Sciences Centre in Toronto, an Assistant Professor at the University of Toronto, a Past President of the Canadian Spine Society that his practice involves the conservative and surgical management of patients with spinal conditions, that he sees a large number of spinal trauma patients whose pathology ranges from minor soft tissue injuries that are self-limiting to catastrophic spinal injuries requiring surgery as well as serious spinal cord injuries, that virtually all of the patients seen in his elective practice are suffering from chronic pain, that he is familiar with somatic symptom disorder and is involved in teaching medical students, residents and fellows with respect to this specific condition known as chronic pain syndrome or more recently somatic symptom disorder;

12) The CV of Dr. Clark, who the Moving Defendants have requested conduct the proposed Physiatry DME states that his clinical practice deals with Electromyography and the diagnosis of neuromuscular disease including peripheral nerve entrapments, radiculopathies, peripheral neuropathies, motor neuron disease and myopathies, that a large portion of his referrals are related to motor vehicle accidents of work place injuries, that he routinely treats back pain neck pain, shoulder pain, osteoarthritis, and neuropathic pain. Most referrals have chronic pain of some variety, that he is trained in the application of the 4th AMA Guides and has been writing physiatry CAT reports and executive summaries for 10 years, is a physiatry consultant at Joseph Brant Memorial Hospital, was a member of the Combined Spine Clinic for 5 years and was the Medical Director of the Rehabilitation Ward for five years, where he treated arthroplasty, stroke, deconditioning amputees, acquired brain injury, post-fracture and spinal cord

injuries, and has a fellowship in Physical Medicine and Rehabilitation with a designation in Pain Medicine.

Necessity

[68] The *Godin* necessity test requires the court to consider whether the Moving Defendants have demonstrated that the requested medical examination is warranted and legitimate, and not made with a view to delaying the trial, causing prejudice to the plaintiffs, or simply corroborating an existing medical opinion, as stated by Ryan Bell, J. in *Klinck* citing D.M. Brown J., as he then was, in *Bonello v. Taylor*, 2010 ONSC 5723 (“**Bonello**”), and *Bonello* is cited in the Facta of both the Plaintiff and the Moving Defendants.

[69] The Moving Defendants must establish that the requesting examination is likely to produce information relevant to the action which usually requires evidence about the purpose of examination or medical testing, and how it relates to a medical issue in the action

[70] In my view, this element favors the Moving Defendants with respect to the requested Physiatry DME. The Plaintiff has obtained and served the extensive Sangha Physiatry Report from an expert with similar Physical Medicine, Pain and Rehabilitation qualifications as Dr. Clark, the proposed expert for the Moving Defendants.

[71] The Moving Defendants have not provided any affidavit evidence by an expert on this motion specifically as to the requirement for a Physiatry DME. However, both the Moving Defendants in the Mallia Affidavit, and the Plaintiff in the Gilbert Affidavit as well as in the Sangha Physiatry Report, have provided cogent medical evidence that the source, cause and severity of the Plaintiff’s pain and the Plaintiff’s prognosis of future rehabilitation will be crucial issues at the trial, and as a result, I find that expert evidence on these medical issues at trial is warranted and legitimate, for the benefit of both for the Plaintiff and the Moving Defendants.

[72] The Plaintiff states the following in the Gilbert Affidavit:

“21. On December 1, 2021, Mr. Lakdawala attended the DME with Dr. Ford. In his report dated February 3, 2022 (which was not served on Mr. Lakdawala until April 7, 2022), Dr. Ford addressed the issue of pain and specifically rebutted the physiatry report of Dr. Sangha. Dr. Ford did not recommend that Mr. Lakdawala undergo a physiatry assessment or neurosurgical assessment.”

[73] In argument and in their Factum counsel for the Plaintiff asserted that the requested Physiatry DME was not necessary because Dr. Ford addressed the issue of pain in the Ford Orthopaedic Report where he also commented on the Sangha Physiatry Report.

[74] Dr. Ford’s comments on the Sangha Physiatry Report consisted of two paragraphs in his 47 page report. He also comments on the reports of Neurosurgeons, the McKee Orthopaedic Report, other Orthopaedic surgeons’ clinical notes and the future care cost report, which will likely make good sport at Trial, but I do not find entirely disentitles the

Moving Defendants from requesting a specific Physiatry DME to respond to the Sangha Physiatry Report.

[75] The qualifications of Dr. Ford, as tediously set out by me above, are entirely different from the qualifications of Dr. Sangha and Dr. Clark who clearly have similar qualifications in their specialty. I find that the Court would be assisted in weighing and balancing the expert evidence of Dr. Sangha, by having the expert evidence of Dr. Clark, from a similar specialty, with similar qualifications, on the issues of source, cause and severity of the Plaintiff's pain and the Plaintiff's prognosis of future rehabilitation.

[76] The Plaintiff states the following in his Factum regarding the lack of expert evidence produced by the Moving Defendants to necessitate a Physiatry DME:

“36. As stated recently by Jolley A.J., while dismissing a Defendant's request to compel the Plaintiff to attend a 4th DME: While the Defendant argued that these defence medical examinations were "generally" granted, the law is clear that there must be sufficient evidence to persuade the court that a further examination should be ordered.

Rocca v. 6131646 Canada Inc., 2021 ONSC 8445 at para. 21 (CanLII) [Rocca].

37. It is generally expected that the evidence will from the proposed DME expert. While not mandatory, the failure to do so often proves fatal to the motion, as it should in this case.

38. In the recent Rocca decision, the Plaintiff attended an orthopaedic DME but the defence also sought to compel her to attend a physiatry DME (with Dr. Clark, no less). The absence of evidence filed by Dr. Clark was part of the Court's basis for dismissing the motion. Jolley A.J. stated:

“Further, there is no evidence before me from Dr. Clark - or anyone else - concerning what sort of an examination he would undertake and how that would differ from that done by Dr. Marks. The only information I have is from a lawyer.”

Ibid. at para. 19.

39. Similarly, in *Wice v. Dominion of Canada General Insurance Co.*, dismissing an appeal of an Order that dismissed a request for a DME, McEwan J. stated: ... I am struck by the fact that no affidavit evidence was adduced at the original motion by Dr. Gerber; nor is there any evidence the plaintiff has had a similar in-house assessment with his own psychologist. ... Eberhard J's decision is clearly influenced by the fact that no expert evidence was provided to her in support of the need for such an occupational therapist report.

Wice v. Dominion of Canada General Insurance Co ., [2009] O.J. No. 5607 (Sup.Ct.) at paras. 18, 32 [Wice].

40. In *Woolsey v Industrial Alliance Insurance and Financial Services Inc .*, this Honourable Court refused to compel the Plaintiff to submit to a neuropsychological examination where none of his treatment providers had expressed concerns with his cognitive functioning or recommended a neuropsychological assessment. Dismissing the motion, Rady J. stated: Third, there is no suggestion that any of the defence assessors have considered such an examination is necessary. There is only Ms. Larivière's conclusion. As already noted, she is a technical advisor with the defendant, and as far as I know, she has no medical background except that which is incidental to her employment.

Woolsey v Industrial Alliance Insurance and Financial Services Inc., 2016 ONSC 7617 at para. 20 (CanLII) [Woolsey].

41. In *Ramrup v. Lazzara*, after the Plaintiff's Factum emphasized the absence of evidence from the proposed DME experts, the moving party filed a supplementary Affidavit by counsel which, this time, attaching letters from the proposed DME experts as exhibits. Dismissing the motion, Mitrow J. stated:

“I find that the defendant's failure to include some evidence from the experts on this issue when the defendants' motion was initially served, impacts adversely on the defendants' arguments. Although evidence from a proposed defence expert is not mandatory, it is one factor to consider on the facts of the present case. I further find that the hastily prepared responses attached to the supplementary affidavit do little to assist the defendants ...

Ramrup v. Lazzara, 2014 ONSC 130 at paras. 78-79 (CanLII) [Ramrup].”

[77] The Plaintiff is correct that the provision of expert evidence is not mandatory, but is persuasive. In the cases above, clearly in *Rocca* and *Wice*, possibly *Woolsey* these were situations where the Plaintiff had not previously obtained an expert report in a particular discipline and the Defendants were trying to prove the necessity of obtaining one.

[78] In *Ramrup*, the request for not for a new DME, but rather that further Physiatry and Psychiatry DME's be ordered, after the Plaintiff had already been examined by those experts at least once, and there had been a total of 7 reports from the Plaintiff's experts, and 6 reports from the Defendant, three from each defence expert. That is not the case here where the Plaintiff has obtained the single Sangha Physiatry Report, that the Moving Defendants are seeking to respond to by obtaining their own Physiatry DME.

[79] With respect to the necessity of an in-person DME as opposed to a documentary review, having read the Sangha Physiatry Report, and the methodology used by Dr. Sangha in determining the Plaintiff's subjective perception of his pain, I cannot see how a documentary review will permit a similar determination by the Moving Defendant's expert

Dr. Clark, and I find that in person DME would be necessary to provide the Trial Judge with an apples-to apples comparison of the two Physiatry expert reports.

[80] The requested Physiatry DME is not being sought by the Moving Defendants to corroborate an existing medical opinion, as the Moving Defendants have not obtained a Physiatry opinion to date. Having read the Ford Orthopaedic Report, I do not perceive the intent here of the Moving Defendants as corroborative, but rather responsive to the Sangha Physiatry Report.

[81] I also cannot determine from the evidence before me about a specific intent by the Moving Defendants to delay the trial through their request to obtain the Physiatry DME. The parties have produced much correspondence from the attempts to schedule the Physiatry DME, commencing with the apparent first request on October 29, 2020, proceeding fitfully during the period November 2020 to September 2021 when the Plaintiff could not return to Canada, and the request for the Physiatry DME was rejected for the first time, as far I can discern from the evidence, by the letter from Gilbert on October 27, 2021, a year after the Physiatry DME was first requested, and after several cancelled Physiatry DME appointments.

[82] There is much discussion in the Gilbert Affidavit, and exhibits thereto about alleged bungling by the Moving Defendants in scheduling motions and hearings, and alleged unresponsiveness to requests to set timetables and schedules for exchange of experts reports, leading to the interventions of D. Wilson, J. and Glustein, J. However, I cannot find on the evidence before me a specific intent to delay the trial, and do not do so.

[83] In my view, the requested Physiatry DME is likely to produce information relevant to the Plaintiffs' claim, and the necessity test favours the Moving Defendants.

Fairness

[84] As stated by Ryan Bell, J. in *Klinck*, citing *Godin*, at paras. 24-25, the policy behind defence medical examinations is to allow the defendant to meet the plaintiff's case and to respond to allegations in the statement of claim and to assist the trial court by providing expert evidence that is subject to the adversarial process.

[85] I am very cognizant of the oft-cited quotation of D.M. Brown, J. (as he then was) in *Bonello* as quoted by Ryan Bell, J. in *Klinck*, and *Bonello* is cited by both the Plaintiff and the Moving defendants on this Motion:

[79] The importance of ensuring trial fairness in the context of a request for a further examination was highlighted by D.M. Brown J., as he then was, in *Bonello v. Taylor*, 2010 ONSC 5723, at para. 16:

“That said, I would venture that trial fairness should operate as the guiding principle in this area, so if the plaintiff has decided that expert evidence from one specialty based on an examination of the plaintiff is relevant to the adjudication of her claim at trial, courts should be loathe to deny the

defence a fair opportunity to respond with expert evidence from the same specialty based on an assessment of the plaintiff. Ordering further examinations may be just where they are necessary to enable the defendant fairly to investigate and call reasonable responding evidence at trial.”

[86] I am also persuaded by the following reasoning of AJ McGraw in *Chandrababu*:

“[21] The purpose of a defence medical examination is to achieve trial fairness and a level playing field by putting the parties on a basis of equality as near as it is possible (*Ortiz* at paras. 2-5; *Rohit* at paras. 24-29; *Vanderidder v. Aviva Canada Inc.*, 2011 CarswellOnt 8811 at para. 35). Allowing a plaintiff to adduce expert evidence of damages while denying the defendant the ability to test and challenge that evidence through its own expert is inconsistent with trial fairness, unduly restricts the defendant’s pre-trial preparation and risks creating a perceived imbalance (*Moore v. Wakim*, 2010 ONSC 1991 at para. 4; *Vanderidder* at para. 35).”

[87] On the evidence above and for the reasons discussed in the Necessity section, I find that to meet the Plaintiff’s case and to respond to allegations in the statement of claim, and to assist the trial court by providing expert evidence that is subject to the adversarial process, an in-person Physiatry DME is required to permit the Moving Defendants to fairly investigate the Plaintiff’s substantial claims against them and to call reasonable responding evidence at trial, and it would also be unfair to the Moving Defendants to deny them a fair opportunity to respond to the Sangha Physiatry Report with expert evidence from the Physiatry specialty based on an assessment of the Plaintiff, as per *Bonello* and *Chandrababu*, and also unfair to the trial court to determine the Plaintiff’s claim without the benefit of this evidence.

Proportionality

[88] Given the interpretation by Perrell, J. in *Rothmans* that:

“The general principle in interpreting the rules set out in subrule 1.04(1) requires that the rules be liberally construed to secure the just, most expeditious and least expensive determination of every civil proceeding on its merits. Subrule 1.04(1.1), subtitled "Proportionality", was added which states: In applying these rules, the court shall make orders and give directions that are proportionate to the importance and complexity of the issues, and to the amount involved, in the proceeding”

I find that the ordering of the Physiatry DME would not offend this Rule, as on my review of the evidence before me, on the fairness tests of *Bonello* and *Chandrababu* set out above, it “just” that the Physiatry DME be ordered, to respond to the Sangha Physiatry Report, and ordering the Physiatry DME is proportionate to the importance and complexity of the issues in this Action and to the amount involved in the proceeding and does not offend the parsimonious principle of discovery espoused by Perrell, J. in *Rothmans*.

Prejudice

[89] On the issue of Prejudice, the Plaintiff opposes the granting of the Psychiatry DME on the basis that the granting of this relief may prejudice the Trial Date in May of 2023 in the schedule set by Glustein, J., stating in his Factum:

“46. Notably absent from the Moving Defendants' Factum are any cases regarding the issue of the timing of the request for the DME. Indeed, the issue of timing is highly relevant to the disposition of the motion.

Babcock, supra at para. 13.

47. The Moving Defendants have noted in their Factum that compelling Mr. Lakdawala to attend DMEs in June of 2022 will not result in any prejudice to him as the pre-trial is not scheduled until February of 2023. What they have failed to address is the time that would reasonably be needed thereafter to allow Mr. Lakdawala to obtain a responding neurosurgical opinion and/or to have his existing experts (including, but not limited to his future care expert), consider the opinions of Drs. Fazl and Clark.

Factum of the Moving Party at para. 32.

48. In *Stetler v. Christmas*, the Plaintiff, who was alleged to have suffered a mild brain injury and psychological dysfunction, served a psychological report in October of 2018. Days later, the defence requested that she attend both psychological and neuropsychological DME's. The Plaintiff agreed to attend the psychological DME, but not the neuropsychology DME on the basis that she had not retained any such expert. Around the same time, in November of 2018, the pre-trial and trial were scheduled for May 2019 and September 2019, respectively. Dismissing the motion in March of 2019, Thomas RSJ. stated: "I am concerned that a defence neuropsychological assessment will simply prompt Plaintiff's counsel to respond in kind and abort the schedule presently set." *Stetler v. Christmas*, 2019 ONSC 1616 at para, 18 (CanLII) [Stetler].

49. In *Suida v. Cunai*, dismissing a motion for a DME, Emery J. stated:

“Prejudice can take the form of a trial put in jeopardy of being delayed or postponed ... I find that the plaintiff will suffer prejudice by having her trial date pushed back any further. The defendant has not provided a reasonable explanation ...”

Siuda v. Cunaj. 2014 ONSC 2069 at paras. 26, 28 (CanLII) [Siuda].

50. In *Daggitt*, dismissing the DME motion brought five months before trial, Macleod-Beliveau J. stated:

“Seeking such an order at this late stage of the proceedings with a September 2016 trial date is all but guaranteed to delay the upcoming trial. Regardless of the looming trial date, the defendants simply do not need an assessment by a psychiatrist to defend this case ... *Daggitt v. Campbell*, [2016] O.J. No. 2226 (Sup.Ct.) at paras. 19 [*Daggitt*],”

56 As evidenced by the cases cited above, the issue of prejudice is very much tied to the issue of timing. The Court must balance the rights of and/or potential prejudice to each party. Doing so requires a careful consideration of the evidence filed.

57. The absence of evidence of prejudice was a significant factor in *Ananthamoorthy v. Ellison*, where, dismissing a DME motion, Stinson J. stated:

“Also noteworthy is what is lacking from the defence evidence: any proof of prejudice. There is no evidence whatsoever in the affidavit to support the conclusion that the defendants will somehow be prejudiced or put in an unfair position if the order sought is not granted. While the defendants would like to advance that argument, it must be founded upon an evidentiary base that can be assessed and weighed by the court. Here there is nothing to assess or weigh. There is no evidence in the present case from any defence doctor or any doctor at all to support the conclusion that a further medical examination is required to allow the defendants to present their case.”

Ananthamoorthy (Litigation guardian of) v. Ellison, 2013 ONSC 340 at paras. 22 and 30 (CanLII) [*Ananthamoorthy*].”

[90] I agree with the Courts in *Stetler*, *Suida* and *Daggitt* that if the proposed Psychiatry DME in this case threatened the case timetable in this Action, and the May 2023 trial date, it should not be permitted, as this would prejudice the Plaintiff.

[91] The Moving Defendant’s evidence on prejudice is meagre in the Mallia Affidavit, but I am also guided by the tests in *Bonello* and *Chandrababu*, and the Moving Defendants have provided some evidence in the Mallia Affidavit that the inability to respond to the Sangha Psychiatry Report will be prejudicial, and the *Bonello* and *Chandrababu* tests are very persuasive to me that it would in fact be prejudicial to the Moving Defendants to proceed without a responding Psychiatry DME in these circumstances, and on the evidence before me.

[92] However in this case the Psychiatry DME is scheduled with Dr. Clark on June 14, 2022. Glustein J. has expedited this hearing to be heard before June 10, 2022 in the Glustein, J. Endorsement and I have expedited these reasons to also meet that date. Under R. 33.01 I may order the Psychiatry DME “...on such terms respecting costs and other matters as are just”.

[93] As noted in the Plaintiff’s Factum excerpts above, and in argument before me, the Plaintiff’s concern is that Dr. Clark and counsel for the Moving Defendants will be

dilatory in providing their Psychiatry Report to the Plaintiff, and as a result the Plaintiff will not be able to have its experts provide reply reports in time to meet the Pre-Trial date of February 28, 2023 and to have those reply reports filed within the strictures of R.53.03(1).

[94] Given that the Judicial system has made accommodations to the Moving Defendants, several times, including the hearing of this motion on an urgent basis, I can solve this problem by Ordering that the Moving Defendants and Dr. Clark also accommodate the Court and the case timetable in this Action by requiring that the report of Dr. Clark be provided to counsel for the Plaintiff by the Moving Defendants within 45 days of the June 14, 2022 examination date, which would be before the end of business on July 29, 2022. By my count that would leave 214 days before the pre-trial date, with a R.53.03(1) date of November 30, 2022, and 124 days for the Plaintiff to prepare reply and serve experts reports before the R.53.03(1) date.

[95] Having considered all of the factors described in the *Godin* test of Necessity, Fairness, Prejudice and the additional test of Proportionality, and exercising my discretion as required under R.1.04, I DO grant the Order requested by the Moving Defendants for a Psychiatry DME.

[96] With respect to the terms of the Order requested by the Moving Defendants, I will Order:

“THIS COURT ORDERS that the Plaintiff attend and complete a psychiatry medical examination with psychiatrist Dr. Benjamin Clark on June 14, 2022, from 9:30 a.m. to 11:30 a.m., at 5945 Airport Road, Suite 335, in the City of Mississauga, in the Province of Ontario and that the report prepared by psychiatrist Dr. Benjamin Clark shall be served upon counsel for the Plaintiff no later than the close of business on July 29, 2022.”

[97] I WILL NOT Order the following language requested in the draft Order:

“failing which the moving defendants may move, without notice, to dismiss or discontinue the within proceeding as against them”

which is inappropriate in the circumstances of the Moving Defendants receiving significant accommodation(s) from this Court, and where this matter is subject to strict timetabling by the Court through case conferences, and Glustein, J. has seized himself with this scheduling.

Analysis: Request for Neurosurgical DME:

The arguments raised above by the Plaintiff are more resonant to the request for a Neurosurgical DME.

Factual matrix for request for Neurosurgical DME:

[98] The facts and chronology relevant to the request for a Neurosurgical DME are different than request for the Physiatry DME, and attached correspondence in the Gilbert Affidavit and the Mallia Affidavit I find the following facts as relevant to my determination:

- 1) No Neurosurgical report has been obtained by the Plaintiff and none is being sought;
- 2) The Plaintiff was last seen by a Neurosurgeon with respect to his collision injuries in November 2015, and the Clinical Note provided appears to indicate he requires no Neurosurgery follow-up;
- 3) An unsuccessful mediation was conducted on June 11, 2018;
- 4) The Examination for Discovery of the Plaintiff was conducted on November 29, 2018;
- 5) The Plaintiff is seen by a Neurosurgeon in 2019 in relation to an apparently benign frontal brain tumor;
- 6) Updated clinical notes for both issues were provided to counsel for the Moving Defendants on or about January of 2019;
- 7) This Action was set down for Trial by the Plaintiff on March 4, 2020;
- 8) The first mention by the Moving Defendants of a third DME was on June 24, 2021, but that email from Mr. Biniaz does not appear to state specifically that this was a request for a Neurosurgical DME;
- 9) In an email dated August 13, 2021 at Exhibit S to the Mallia Affidavit Mr. Biniaz specifically identifies the third requested DME as a “neurosurgical” DME in the course of counsel exchanging dates for the service of expert reports;
- 10) The Plaintiff was out of the country from November 25, 2020 until September, 2021, in part prevented from returning to Canada by Government of Canada COVID travel restrictions;
- 11) The Plaintiff, through counsel, first refused to attend Physiatry and Neurosurgical DME’s on October 27, 2021, but agreed to attend the Orthopaedic DME;

12) The Orthopaedic DME agreed to by the Plaintiff occurred on December 1, 2021 and the Ford Orthopaedic Report was issued on February 3, 2022 and served on about April 7, 2022;

12) The CV of Dr. Fazal, who the Moving Defendants have requested conduct the proposed Neurosurgical DME states that he has a fellowship in Neurosurgery and that he is an Assistant Professor at the University of Toronto and currently the Director, Neurosciences Course for 3rd year medical students, and Director of Spinal Cord Injury Unit at Sunnybrook Health Science Centre as well as having cross-appointments to Lyndhurst Hospital and Toronto Bayview Clinic.

Necessity

[99] As noted above the *Godin* necessity test requires the court to consider whether the Moving Defendants have demonstrated that the requested medical examination is warranted and legitimate, and not made with a view to delaying the trial, causing prejudice to the plaintiffs, or simply corroborating an existing medical opinion, as stated in *Klinck* and *Bonello*.

[100] The Moving Defendants must establish that the requested Neurosurgical examination is likely to produce information relevant to the action which usually requires evidence about the purpose of examination or medical testing, and how it relates to a medical issue in the action

[101] In my view, this element DOES NOT favour the Moving Defendants with respect to the requested Neurosurgical DME. The Plaintiff has not obtained a Neurosurgical expert report and is not seeking one.

[102] There is no cogent medical evidence before me that contradicts that the Plaintiff has had no follow up Neurosurgical treatment with respect to his Collision injuries since 2015, as set out in the Gilbert Affidavit and exhibits thereto. There is also no evidence before me that contradicts the statements in the Gilbert Affidavit that the frontal tumor is benign and that no further treatment was obtained by the Plaintiff since 2019. Accordingly I do not find that expert evidence on this issue at trial is warranted and legitimate.

[103] I find that the *Rocca*, *Wice* and *Woolsey* cases cited above are persuasive in the context of the requested Neurosurgical DME as the onus is on the Moving Defendants to provide evidence, expert or otherwise, that the medical condition of the Plaintiff necessitates the ordering of a Neurosurgical DME, and the following quotation by AJ Jolley in *Rocca* is relevant here:

“While the Defendant argued that these defence medical examinations were “generally” granted, the law is clear that there must be sufficient evidence to persuade the court that a further examination should be ordered.”

[104] I find that based on the evidence before me in the Mallia Affidavit, which I have expressed reservations of above from an evidentiary perspective of a law clerk providing

expert evidence, that the Moving Defendants have not provided sufficient evidence, expert or otherwise, to persuade me that a Neurosurgical DME should be ordered and, in my view, the Moving Defendants have not provided sufficient evidence to discharge their onus of proving that the requested Neurosurgical DME is likely to produce information relevant to the Plaintiffs' claim, and as a result the necessity test favours the Plaintiff.

Fairness

[105] As stated in *Klinck*, citing *Godin*, the policy behind defence medical examinations is to allow the defendant to meet the plaintiff's case and to respond to allegations in the statement of claim and to assist the trial court by providing expert evidence that is subject to the adversarial process.

[106] Unlike the case with the request for a Psychiatry DME, the *Bonello* and *Chandrababu* tests are not relevant to this request, as the Plaintiff has not previously obtained a Neurosurgical report and will not be doing so prior to trial. There is no fairness issue of not being able to respond to the Plaintiff's Neurosurgical evidence, as the Plaintiff is not leading this evidence. The Plaintiff merely making a claim of "cognitive impairment" in the Statement of Claim is insufficient to ground a request for a Neurosurgical DME.

[107] The evidence of the Plaintiff is that the Moving Defendants have been aware since January 2019 of the Neurosurgical consult that the Plaintiff attended in 2015 and 2019, and appear in the evidence before me to have first requested the Neurosurgical DME by name in August of 2021. There was ample time to raise these issues prior to that time.

[108] In determining the Fairness test in *Godin*, and on the evidence stated above, and for the reasons discussed in the Necessity section, I DO NOT find that a Neurosurgical DME is required to permit the Moving Defendants to fairly investigate the Plaintiff's claims against them, to meet the Plaintiff's case, to respond to allegations in the statement of claim and to assist the trial court by providing expert evidence that is subject to the adversarial process.

Proportionality

I find that not ordering the Neurosurgical DME would not offend this Rule, as on my review of the evidence before me, it "just" that the Neurosurgical DME not be ordered.

Prejudice

[109] On the issue of Prejudice, the Plaintiff opposes the granting of the Neurosurgical DME on the basis that the granting of this relief may prejudice the Trial Date in May of 2023

[110] I agree with the Courts in *Stetler*, *Suida* and *Daggitt* that if the proposed Neurosurgical DME in this case threatened the case timetable in this case, and the May 2023 trial date it should not be permitted, as this would prejudice the Plaintiff.

[111] As noted previously, the Moving Defendant's evidence on prejudice is meagre in the Mallia Affidavit, and the tests in *Bonello* and *Chandrababu* are not a factor here, and the Moving Defendants have provided no cogent evidence that the inability to conduct the Neurosurgical DME will be prejudicial.

[112] Unlike the Psychiatry DME, in this case the Plaintiff would have to retain its own Neurosurgical expert to respond to the Moving Defendants' Neurosurgical DME, and then the Moving Defendant's would have an opportunity to reply.

[113] The Plaintiff argues that there is insufficient time for the Moving Defendants to provide the Neurosurgical Report to the Plaintiff, for the Plaintiff to then retain his own expert to do a responding report, and to then allow the Moving Defendant to provide a reply report, in time to meet the Pre-Trial date of February 28, 2023 and allow all these reports to be filed within the strictures of R.53.03(1).

[114] As I calculated above, I agree with the Plaintiff that it would be unlikely that all of these tasks could be accomplished before the February pre-trial date, and within the R.53.03(1) date of November 30, 2022, and as a result I find on the evidence before me that the Plaintiff will likely be prejudiced if I ordered the requested Neurosurgical DME.

[115] Having considered all of the factors described in the *Godin* test of Necessity, Fairness, Prejudice and the additional test of Proportionality, and exercising my discretion as required under R.1.04, I DO NOT grant the Order requested by the Moving Defendants for a Neurosurgical DME.

Costs:

[116] If the parties cannot agree on the disposition of the costs of the motion, they may make written submissions, not exceeding three pages each, the Moving Defendants within 20 days, and the Plaintiff within 40 days.



Associate Justice Ilchenko
Superior Court of Justice

May 20, 2022

SCHEDULE “A”

DISPOSITION OF UNDERTAKINGS CHART

**ONTARIO
SUPERIOR COURT OF JUSTICE**

B E T W E E N:

MOHAMED LAKDAWALA

Plaintiff

and

AARON LIPTON, ERIN JOHNSTON, AVIVA INSURANCE
COMPANY OF CANADA, ~~TAVERN COMPANY 2439004~~
ONTARIO INC. operating as CAVA, ELKHOUND INVESTMENTS
INC. operating as SCALLYWAGS BAR AND RESTAURANT,
JOHN DOE 1, JOHN DOE 2, and JOHN DOE 3, JOHN DOE 4,
JOHN DOE 5, and JOHN DOE 6

Defendants

2022 ONSC 7381 (CanLII)

UNDERTAKINGS CHART

UNDERTAKINGS						
Examination for Discovery of the Plaintiff, Mohamed Lakdawala, held on November 29, 2018						
	Issue & relationship to pleading or affidavit (Group the undertakings by issues)	Ques tion No.	Page No.	Specific Question	Date answered or precise reason for not doing so	Disposition by the Court
1.		425	63	To provide his new passport with the stamps up to trial MR. KAZDAN: Okay, that helps. And for the new passport, can we also get that before trial? MR. MANDEL: Yes.	OUTSTANDING	If a new passport is obtained by the Plaintiff prior to trial, Counsel for Plaintiff has undertaken to provide it prior to trial. No

						order compelling is required.
2.		521	82	Mr. Lakdawala's tax returns and assessments from Canada Revenue from 2012 on MR. KAZDAN: Ask for some productions. We'll get the updated tax returns and assessments up to trial, I take it? MR. MANDEL: Yes.	ANSWERED / ONGOING Counsel produced Mr. Lakdawala's income tax returns from 2008 to 2017 March 2, 2021: Counsel produced Mr. Lakdawala's income tax returns from 2018 to 2019 April 12, 2022: Counsel produced Mr. Lakdawala's income tax returns for 2020 OUTSTANDING: Updated income tax returns from 2021 to Trial	Plaintiff is not currently in breach of obligations regarding answering undertakings under R.31.07. no Order compelling is required. Counsel for Plaintiff has agreed to provided 2021 return and Notice of Assessment when it is assessed.
3.		896	139	Updated records of St. Michael's Hospital including the head injury clinic and including Dr. Vaidyanath and the diagnostic images MR. KAZDAN: Okay. If we don't have the -- I guess we'll need the updated records from St. Michael's Hospital, including the head injury clinic. MR. MANDEL: Yes. MR. REIMER: That will include Dr. Vaidyanath? MR. MANDEL: Yes, it does. MR. KAZDAN: Yes. Can we also get the diagnostic images? MR. MANDEL: Yes.	ANSWERED / ONGOING Jan 23, 2019: Counsel produced records under cover December 3, 2018 May 27, 2021: Counsel produced records from September 8, 2020 to September 22, 2020	Plaintiff is not currently in breach of obligations regarding answering undertakings under R.31.07, No Order compelling is required. Counsel for Plaintiff has agreed to

					April 12, 2022: Counsel produced correspondence stating there were no records from September 23, 2020 to March 10, 2022 OUTSTANDING: Updated records from March 10, 2022 to Trial	request further records if Plaintiff reattends.
4.		896	140	Clinical notes and records of Dr. Khan MR. KAZDAN: We'd better get the clinical notes and records of Dr. Khan. MR. MANDEL: Yes.	ANSWERED / ONGOING February 1, 2019: Counsel produced records from July 16, 2015 to January 17, 2019 Mar 14, 2022: Counsel advised that records were provided on February 14, 2022 for the period of January 17, 2019 to June 30, 2021, but we have not received same OUTSTANDING: Updated records from January 17, 2019 to Trial	Counsel for Plaintiff has agreed to resend these clinical notes and records from Dr. Khan that she advises were provided on February 14, 2022 for the period of January 17, 2019 to June 30, 2021. No Order compelling required.
5.		896	140	Updated records of Toronto Rehab including his therapy records (physiotherapy) MR. KAZDAN: If there are updated records from Toronto Rehab can we get those, please? MR. MANDEL: I don't know if there are, but I will simply write and ask for updated records.	ANSWERED / ONGOING August 17, 2021: Counsel produced records from Toronto Rehab from January 29, 2019 to July 2,	Counsel for Plaintiff has agreed to send another letter to Toronto Rehab, copied to

				MR. KAZDAN: Including the -- I don't think we have his therapy records. Can we get those, please? I know he got physiotherapy there. MR. MANDEL: Yes	2021 OUTSTANDING: Updated records from July 2, 2021 to Trial	counsel for the Moving Defendants, to provide updates to Toronto Rehab's clinical notes and records. No Order compelling required.
6.				Clinical notes and records of Dr. Barbera Q. And Dr. Barbera, B-A-R-B-E-R-A?	ANSWERED / ONGOING August 17, 2021: Counsel produced records from July 11, 2019 to June 30, 2021 OUTSTANDING: Updated records from June 30, 2021 to Trial	Counsel for Plaintiff has agreed to send another letter to Dr. Barbera, copied to counsel for the Moving Defendants, to provide updates to Dr. Barbera's clinical notes and records. No Order compelling required.
7.		913	143	Clinical notes and records of Dr. Adam, therapist for massage and physiotherapy in India for 3 years pre-accident on MR. KAZDAN: --i. And the Dr. Mr. Adam is your therapist. Mr. Adam. THE DEPONENT: Mm-hmm. MR. MANDEL: Three years pre-crash? MR. KAZDAN: Yes, please.	OUTSTANDING August 27, 2021: Counsel advised that they sent productions but we have not received same OUTSTANDING: Please resend	Counsel for Plaintiff has agreed to resend these records. No Order compelling required.
8.		913	144	The updated OHIP decoded summary	ANSWERED / ONGOING	Counsel for

				MR. REIMER: Also, can we get the OHIP decoded summary from three years prior? MR. MANDEL: You have it. You have it, there's no attendances, zero. MR. KAZDAN: Can we get the updated, please? MR. MANDEL: Yes	Feb 21, 2019: Counsel produced records from December 7, 2016 to January 31, 2019 March 15, 2022: Counsel produced records from January 31, 2019 to October 26, 2021 OUTSTANDING: Updated decoded OHIP summary from October 26, 2021 to Trial	Plaintiff has agreed to send another letter to OHIP, copied to counsel for the Moving Defendants to update the OHIP decoded summary. No Order compelling required.
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